Scheduling Timeframe Guide

Trying to find a term listed below quickly? Use **Ctrl F** to find any similar wording

This is guidance for the Patient Services team to identify how quickly a patient needs to be seen or triage direction for common OB/GYN care concerns.

- **If unable to get patient in within the timeframe** listed below with their own Provider or at their home clinic/site, offer other Providers outside of the clinic/hub.
- For visits that are indicated **to be scheduled within 7 days,** and an appointment is not available, the PS team has permission to schedule in a Triage visit spot but should still be scheduled within the timeframe listed below.
- If still unable to schedule per the guidance below or **you need further guidance**, see the "Connecting with a Nurse" chapter at the end of this document.

GYN Visits – Established Patients:

Light Red at the top = GYN Triage visit type scheduled by PS

Orange at the top = Est Office Visit in Triage spots as needed by PS team

Patient Visit Type	Provider Type	Scheduling Timeframe	Visit method & Length
Breast lump (gynecology)	APC or Physician	Within 7 days	In person – 30min
Postmenopausal Bleeding	Physician	Within 7 days	In person – 30min
Pessary Concerns (ie Pain, pressure, bleeding)	Use Pessary Subgroup	Within 24 hours or advice for evaluation	In person – 30min
Pregnancy termination: Options counseling	APC or Physician	Within 48 hours	Telehealth or in person – 30min
Emergency Contraception	APC or Physician	Within 48 hours	Telehealth or in person – 30min
Vaginitis symptoms	APC or Physician	Within 72 hours	Telehealth – 15min APC In person – 30min MD In Person – 15min
IUD concerns	APC or Physician	Within 72 hours	In person – 30min
Vulvar pain/lump/lesion	APC or Physician	Within 48 hours	In person – 30min
Gyn UTI	APC or Physician	Within 24 hours or advice for evaluation	Telehealth or In person – 15min

Abnormal Uterine Bleeding *If heavy bleeding and hemorrhaging needs to speak to advice (soaking pad in < 1 hr)*	APC or Physician **Refer to postmenopausal bleeding section for postmenopausal patients*	Within 30 days	Telehealth or In person – 30min
Colposcopy	Use Colposcopy subgroup	Within 30 days	In person - 30min
Contraception (new start, counseling, changes) *for emergency, see above	APC or Physician	Within 30 days	Telehealth or in person - 30min
ER Follow Up – *PS Team to Send an* ***Urgent*** *Telephone Encounter Utilizing the ER Follow Up SmartPhrase*	Advice to Evaluate After Reviewing Encounter & ER Chart Note	Advice to Evaluate After Reviewing Encounter & ER Chart Note	Advice to Determine – 30min
Fertility consults	APC or Physician	Within 90 days	Telehealth or In person – 30min
LEEP	Use LEEP subgroup	Provider discretion	In person – 30min
Medication New/Consult/Change	APC or Physician	Within 30 days	Telehealth or In person – 30min
Medication Refill	APC or Physician	Follow Smart Phrase	Telehealth or In person – 15min
Menopausal Symptom Discussion/HRT	APC or Physician	Within 60 days	Telehealth or In person – 30min
Mental health-anxiety/depression, suicidal ideation	Advice to evaluate	Advice to evaluate	Advice to determine

Nexplanon Concerns	Use Nexplanon Subgroup	Within 30 days	Telehealth or In person – 30min
Pain with intercourse	APC or Physician	Within 30 days	In person – 30min
Pelvic mass (referral)	Physician	Referral navigator to evaluate or within 14 days	In person – 30min
Pelvic pain	APC or Physician	Within 45 days- advice to evaluate if patient feels their issue is more urgent	Telehealth or In person – 30min
Pessary Management cleaning/replacing/fitting	Use Pessary Subgroup	Send to Gynecology Coordinator or PSR	In person – 30min
Post-operative concerns	Advice to Evaluate *surgeon or physician only	Advice to evaluate	Advice to determine
Preconception counseling	APC or Physician	Within 90 days	Telehealth or In person – 30min
Prolapse- if patient is experiencing an emergency send to advice	Physician or use pessary subgroup	Within 30 days	In person – 30min
Repeat pap	APC or Physician	Within 30 days of due date	In person – 15min
STI screening/testing -if patient reports genital lesion/sore/bump must be same day in person visit or advice to evaluate	APC or Physician	Within 14 days	Telehealth + lab or in person – 30min
Surgical consult	Physician	Within 30 days	Telehealth or In person – 30min

APC Scope Outline:

This applies to all APCs within WHA. Note, clinic only APCs see OBs up to 36w GA

Within Scope – APCs	Within scope for APCs with additional training *Appt type exists in Epic	Outside of Scope – APCs
Abnormal uterine bleeding (postmenopausal schedule with physician)	Colposcopy*	Postpartum BP follow up on meds – review further
Birth control consults	Cryotherapy for genital warts*	Cryotherapy of cervical lesions
Breast lumps/pain	Diaphragms	D&C – dilation and curettage
ED follow up visits *RN assessed/scheduled	Endometrial biopsy (EMB)*	Ectopic pregnancy (known / confirmed)
Endometriosis (initial visit, not if referral or surgery planned)	I&D – incision & drainage*	LEEP
Family planning	Nexplanon inserts & removals*	Patients 12 and under
Fibroids, cysts, or polyps (initial visit, not if referral or surgical consult)	Non-stress Tests (NSTs)*	Post-op/ post-procedural complications
First trimester bleeding evaluation (including pregnancy of unknown location)	Ovulation stimulation (Clomid, Letrozole)	Surgery
Genital warts/ lesions	Pessary problems/fittings*	Twins
HRT – hormone replacement therapy	Vulvar biopsy	
Infertility evaluation (initial visit, not if referral or surgery planned)		
IUD inserts & removals		

Mastitis		
Medication Abortions (opted in)		
Menopause management		
Menstrual concerns/problems		
Mood disorders, initial assessment		
OB visits (Clinic only APCs see OBs <36 weeks GA)		
OB triage *RN assessed/scheduled (Clinic only APCs see OBs <36 weeks GA)		
Options counseling		
Pain with intercourse		
Patients greater than or equal to 13 years old		
PCOS – polycystic ovarian syndrome		
Pelvic pain (initial visit, not if referral or surgery planned)		
Pelvic relaxation (initial visit, not if referral or surgery planned)		
STD Testing		
UTI's		
Vaginal Infections		
Wellness exam		

Nurse Triage – Connecting with a Nurse to ask a question or to connect a patient

Priority Skills/Advice Back Line – Should never be cold transfers. Warm transfers only

**Call the WHA location specific Priority Advice line – *Example: Priority BR Advice*

OB or GYN Concerns	
Shortness of Breath &/or Chest Pain	Warm Transfer to Priority Advice Skill or Advice Backline (wait for the RN to answer, and you can anticipate the RN will take the call from you)
Suicidal Thoughts	Warm Transfer to Priority Advice Skill or Advice Backline (wait for the RN to answer, and you can anticipate the RN will take the call from you)
Staff Call to ask RN a Questions	Priority Advice Skill or Advice Backline: Wait for RN to answer, you can anticipate that the RN will be able to give guidance and answer questions, but may provide direction to transfer to the regular advice skill, as they may be in the middle of a more urgent/complicated issues and not able to take the patient at that moment, but the next available RN will from the regular advice skill
Urgent OB Concerns	
Decreased Fetal Movement	Warm Transfer to Priority Advice Skill or Advice Backline (wait for the RN to answer, and you can anticipate the RN will take the call from you)
Vaginal Bleeding in 2nd or 3rd Trimester	Warm Transfer to Priority Advice Skill or Advice Backline (wait for the RN to answer, and you can anticipate the RN will take the call from you)

Labor or Preterm labor Symptoms	Warm Transfer to Priority Advice Skill or Advice Backline (wait for the RN to answer, and you can anticipate the RN will take the call from you)
Trauma in Pregnancy (ie call or Car accident)	Warm Transfer to Priority Advice Skill or Advice Backline (wait for the RN to answer, and you can anticipate the RN will take the call from you)
Questions of Leaking of Fluid/Gushes of Fluid	Warm Transfer to Priority Advice Skill or Advice Backline (wait for the RN to answer, and you can anticipate the RN will take the call from you)
Severe Abdominal Pain in pregnant person	Warm Transfer to Priority Advice Skill or Advice Backline (wait for the RN to answer, and you can anticipate the RN will take the call from you)
Headache &/or BP concerns after 24wks GA	Warm Transfer to Priority Advice Skill or Advice Backline (wait for the RN to answer, and you can anticipate the RN will take the call from you)
Regular Advice Skill - Cold transferring a patient to the live advice Five9 skill where they can choose to wait in line for the next available nurse or choose for a "callback."	
OB or GYN	
Patient is returning a call to a nurse	**Live Advice Skill** – Any nurse that answers can assist the patient
Patient is requesting to talk to a nurse about a concern they are having	**Live Advice Skill -** RN will be able to further triage patient's concern and determine how urgently they need to be seen, and where that care needs to occur if unable to be evaluated within WHA due to availability (ie ER, urgent care, PCP ect)
Patient is unable to be seen within timeframe and guidance listed for appointments above	**Live Advice Skill -** RN will be able to further triage patient's concern and determine how urgently they need to be seen, and where that care needs to occur if unable to be evaluated within WHA due to availability (ie ER, urgent care, PCP ect)
Post Op Questions/Concerns	**Live Advice Skill**

OB / Postpartum		
	Established Patient	New Patient (Scheduled for OB/Postpartum care but has not been seen yet)
OB with Unknown LMP	Complete the NOB Encounter then route that to the OB Coordinator Pool. Start a new Telephone Encounter (use the dot phrase .WHAUNKNOWNLMP and Pregnancy Questions as the Reason for Call) that documents the patient has an Unknown LMP date and cannot be scheduled for their initial OB visits until further clinical instruction, then route that to the Advice Pool	Same as Established patient
Early OB requesting sooner care	Same day or next day Telehealth with any OB provider. If none are available, transfer to Live Advice Phone Line	Same day or next day Telehealth with any OB provider. If none are available, send Telephone Encounter to the Advice Nurse Pool to call back
Early OB requesting *HCGs or early D/V scan* Blood test or Ultrasound for confirmation of pregnancy	Live Advice Skill – Pt to be made aware that an RN is unable to order these, but can further assess their request/concern	Same as Established patient
OB Dizziness / Fainting	Live Advice Skill	Same as Established patient
OB Swelling / Leg pain	Live Advice Skill	Same as Established patient
Nausea & Vomiting in Pregnancy	Same day Telehealth with any OB provider. If none are available, transfer to Live Advice phone line	Same as Established patient

OB Urinary Symptoms (UTI or Bladder infection)	Same day Telehealth with any OB provider. If none are available, transfer to Live Advice phone	Same as Established patient
OB Positive Covid	Live Advice Skill	Same as Established patient
Early OB / 1st trimester Bleeding or Pain	Live Advice Skill	Same as Established patient
OB New medication request	Same day or next day Telehealth with any OB provider. If none are available, transfer to Live Advice phone	Same as Established patient
OB Medication Refill Request	Live Advice Skill	Next available Telehealth visit with any OB provider, or needs to call prescribing provider to get refill
OB Medication questions	Same day or next day Telehealth with any OB provider. If none are available, send Telephone Encounter to the Advice Nurse Pool to call back	Same as Established patient
OB ER Follow Up	Live Advice Skill	Same as Established patient
OB returning call re: HCG results	Live Advice Skill	Same as Established patient
Other OB Questions/Concerns	Live Advice Skill	Same as Established patient
Postpartum Bleeding Concerns	Same day Visit TH or in person with any OB provider. If none are available, transfer to Live Advice phone	Same as Established patient

Postpartum Breast concerns	Same day Visit TH or in person with any OB provider. If none are available, transfer to Live Advice phone	Same as Established patient
Postpartum Medication Refill	Live Advice Skill	Same as Established patient

Epic Routed Phone Encounter to Nursing Team	
Patients can always opt to leave a message with a PS team member to send a note back to the advice pool for an RN to call them back. It does not necessarily ensure faster access to an RN than transferring to the queue to wait on hold for the next available RN or for a queue call back. Try to avoid this option for urgent concerns. May take up to 48 business hours for return call for non-urgent questions	
GYN ER Follow Ups	Refer to Workflow for appropriate smart phrase: ER Follow-up Visits.pdf
OTHER CALLS – Sent to Non-Advice Locations	
Breast Pump Prescriptions	Telephone encounter to Pod (add to existing/open encounter if applicable)
Surgery *Scheduling* Questions/Status	Surgery scheduling Pool
Billing/Financial Questions	Financial Team

Term of Care Patients > 30 days from time letter sent for Financial Reasons	Financial team to get more info RNs cannot give advice to patients who have been termed of care. Pt to follow up with PCP or other care provider (doesn't include ER follow ups where WHA clinician was on call)
Pharmaceutical or medical reps	Send email to Purchasing dept with contact info
Outside of WHA Clinician to Clinician Consults	See Workflow: Outside provider calling in requesting WHA clinician.pdf
NP GYN patients	RNs unable to provide advice to Non-pregnant new patients –Refer back to PCP, Urgent Care, ER until have established care at WHA
Prior Authorizations	Telephone encounter to Pod (add to existing/open encounter if applicable)
FMLA	Telephone encounter to Pod (add to existing/open encounter if applicable)
Genetic Results & Genetic test inquiries	Telephone encounter to **WHA Genetics Results Pool** (add to existing/open encounter if applicable)
Results	Telephone encounter to Pod (add to existing/open encounter if applicable)
Patient asking for PCP recommendations	Refer to insurance for in network providers

Communicable Disease Questions If a patient states that they have any of the following key words/symptoms relating to Infectious or Communicable Diseases, follow the guide below for next steps with the patient	
Respiratory Symptoms or diagnosis of flu, cold or COVID-19	• **OB Patients:** If patient requests, ok to cold transfer the patient to the **Live Advice Skill** for assessment of symptoms and need for expedited care. • **GYN Patients:** May attend their visit(s) if the patient feels well enough. If they request to reschedule, this can be done per their preference. **Additional Guidance:** Management of Patients with Respiratory Symptoms
Draining wounds, stool incontinence or active infections of MRSA, VRE or C.diff Active TB Rash, Fever/Diagnosis of chickenpox, shingles, measles Bed bugs, untreated Scabies or Lice All other Infectious or Communicable Diseases reported by patient	**Warm Transfer to Priority Advice Skill** If already scheduled – • **Urgent Visits (OB, OB/GYN Triage, ER Follow Up):** Warm Transfer to Priority Advice Skill • **Non-Urgent Visits:** Cold transfer call to the **Live Advice Skill**. Advice team to assist with rescheduling timeframe and/or determining if scheduled visit can be kept.